
CONSULT FOR AFRICA

Osteon Clinics

Organisational Audit: Scope, Method and Timetable

Prepared by Dr Debo Odulana, Founding Partner, Consult for Africa

For Dr Bolarinwa Akinola, FRCS (Tr. and Orth.), and the Osteon Clinics leadership

Issued 15 September 2026 / Fieldwork begins Thursday 17 September 2026

hello@consultforafrica.com / +234 913 813 8553 / consultforafrica.com

Lagos and Abuja, Nigeria



Why we are doing this now

Dr Bola, when we finished the positioning work in July we agreed that you would lead as the surgeon other surgeons refer to, that Osteon would stand behind you as a named centre, and that the way you would win the category rather than argue about it was by publishing what you actually achieve when other people will not. I still believe that is the right strategy. I also think we cannot execute any of it responsibly until we have looked properly at the organisation underneath it, because every one of those moves is a promise about the business and not only about the surgeon.

If we tell the market you take the hard cases, the theatre has to be able to run a long revision list without falling over. If we publish your outcomes, the outcomes have to be recorded somewhere we can defend. If we offer a fixed price, we have to know what a case actually costs you, which means knowing what you pay for an implant and how much theatre time a case consumes. And if we bring associates or a second site into the picture later, everything that currently lives in your head has to exist somewhere else first.

So this audit is not a health check we are performing on you. It is the groundwork for the strategy you already own, and the reason it is broad is that in a clinic of this size the constraint is almost never where the owner thinks it is.

I will also say this plainly, because it is the most likely thing we will find and it is better said now than in a report. In July you told me the problem was the location. I do not think it is, and if I am right then the honest finding will be that the practice is bigger than the company, that a meaningful share of what you earn is earned in somebody else's theatre, and that Osteon carries fixed cost for a surgeon who is frequently somewhere else. That is a solvable problem, but it is a different problem from Amuwo Odofin, and we will not know which of us is right until we have seen the numbers.

Warm regards,

Dr Debo Odulana

The three questions this audit answers

Everything below serves one of these. If a piece of analysis does not, we will not do it.

	The question	Why it decides something
1	What is actually here? What does Osteon earn, what does it cost to keep the doors open, how much of your practice runs through it, and what can the building, the theatre and the team safely carry today	Sets the floor. You cannot sell capacity you do not have, and you cannot price work you have never costed
2	Where does the money and the patient leak out? Between an enquiry and a person on the table there are seven or eight places a patient is lost, and between an invoice and the bank there are several more	This is where growth is cheapest. Recovering patients you already attracted costs far less than attracting new ones
3	Can the promise be kept, and can it be proved? Sterile processing, implant traceability, perioperative safety, escalation when a patient deteriorates, and whether your results exist in a form anyone could publish	The whole positioning rests on this. Transparency is only a weapon if the record survives being looked at

What we will examine

The audit covers the whole organisation. The areas below are weighted for what you actually are, which is a surgeon-owned specialist clinic doing elective arthroplasty and complex trauma across more than one site,



and not a general hospital.

The practice and the company, which are not the same thing

Whether income is earned by Osteon Clinics or by you personally, what the arrangements at Cedarcrest, Diamed and elsewhere are worth, what each of those sites gives you that Osteon does not, and how many of the cases done elsewhere could have been done here. We will build one picture of the whole practice, then show you the part of it Osteon actually owns.

The economics of a case

What a primary knee, a primary hip, a revision and an infected two stage case each earn and each cost, once implant, consumable, theatre time, anaesthetic, staff and overhead are counted. This produces the contribution per case that every pricing, bundling and financing decision afterwards depends on. It is also the number that tells us whether the implant supply lever is worth pulling.

The funnel

Enquiry, consultation, decision to treat, quote, deposit, date, surgery, follow up. We will measure conversion at each step and the time spent between them. In private elective surgery this is usually the largest single finding available, and it is almost never measured.

Theatre and the perioperative pathway

List utilisation, planned against actual start and finish, turnaround between cases, cancellations on the day and why, pre-operative assessment, the anaesthetic service and what happens when it is unavailable, recovery, the surgical safety checklist, prophylaxis, and the written arrangement for transferring a patient who deteriorates to a unit with intensive care.

Sterile services, implants and traceability

Decontamination and sterilisation, autoclave validation and cycle records, whether a sterilised set can be traced to the patient it was used on, loan kit logistics, and whether the lot number of every implanted component is recorded against a named patient in a form that would let you find those patients if a manufacturer issued a recall. For arthroplasty this is the highest consequence part of the audit and we will spend real time on it.

Clinical governance and outcomes

The case record, consent, the operation note, the complication and infection register, follow up completeness, and whether any outcome score exists. We will tell you honestly whether you are in a position to publish, what it would take to get there, and what the published figures would look like if you did.

People, rota and capability

Who is here, on what basis, on which days, what they are licensed to do, what happens when you are operating elsewhere, and what the clinic is allowed to decide without you. Establishment against service is where clinics of this size come apart.

Facility, equipment and power

Rooms, flow, theatre environment and ventilation, imaging and radiation compliance, the generator, gases and water, and the state of the asset base.

Money, systems and reporting

Management accounts against bank, receivables and payer mix, the tariff, what software holds what, and whether the numbers you look at can be relied on.



Brand, referral and demand

Where patients actually come from, which referrers convert, what a referring doctor receives back from you and how long it takes, and what the last two years of marketing spend produced.

Regulatory standing

Facility registration and the category registered under, practitioner licences, radiation, waste, indemnity, device registration and data protection. We check this not to police you but because a growth plan built on an exposure is not a plan.

How we will work

Documents first, then the visit. The information request has gone to you alongside this. We read everything that arrives before Thursday so that the day on site is spent on what paper cannot show.

A tour, then a working day. We walk the building with you, front door to sterile room, then we stay and watch. We want to see a clinic running and, if the list allows, a theatre list from the moment the first patient is sent for.

Ten cases, traced end to end. This is the spine of the method and it is worth explaining. Rather than sampling many documents shallowly, we pick ten real cases from your log, spread across primary, revision, infection and trauma, and follow each one completely: the first enquiry, the clinic note, the consent, the pre-operative assessment, the theatre register, the safety checklist, the implant lot sticker, the sterilisation load record, the operation note, the drug chart, the invoice, the payment, the claim if there was one, the follow up attendances and the outcome today. Ten cases traced completely will tell us more about how Osteon works than a hundred sampled partially, and every gap found this way is specific rather than theoretical.

Four short surveys. Staff, patients, referring doctors, and you and your leadership team. Details below.

Interviews. You, whoever runs the clinic day to day, the theatre and nursing leads, the person who handles money and claims, the front desk, the physiotherapist, and at least one visiting anaesthetist. Thirty to forty minutes each, and nothing anyone says is attributed to them by name.

Counts and spot checks on the day. A physical implant stock count against the record, a sample of autoclave cycle records, the emergency trolley against a standard list, drug expiry on a sample of lines, and a handful of assets checked against the register.

Collaborative throughout. Your team will be told, before we arrive, that this is about how the place works and not about who is at fault. Nobody on the frontline should feel inspected. If anything we do starts to feel like audit by ambush, tell me and we will change it.

The four surveys

Survey	Who it goes to	Length	Anonymous	What it is for
Staff and safety culture	Everyone who works at Osteon, clinical and non-clinical	8 minutes	Yes	How the place actually runs, what routines survive a busy day, whether people can speak up



Survey	Who it goes to	Length	Anonymous	What it is for
Patient experience	Patients seen or operated on in the last 12 months	5 minutes	Yes	Access, communication, the money conversation, recovery, and whether they would send someone
Referring doctor	Every doctor, physiotherapist and clinic that has referred, and a shortlist who could	6 minutes	Attributed, with consent	The growth constraint. What referrers want, what they get back today, and what would make them send more
Leadership direction	You and up to five of your senior people	10 minutes	Attributed	Where each of you thinks the business should go, and where you disagree without knowing it

The referrer survey is the one I care most about. The positioning we agreed depends on a referral pipeline, and to my knowledge nobody has ever asked those doctors directly what they need from you. Their answers will shape the referrer engine more than anything we could infer.

Timetable

When	What happens
Tuesday 15 September	Scope and information request issued. Surveys go live. Point of contact nominated at Osteon
Wednesday 16 September	Documents begin arriving. Staff and patient surveys open. Referrer list agreed with you
Thursday 17 September	On site: tour, observation, interviews, ten-case trace begins, physical counts
Friday 18 to Friday 25 September	Remaining documents. Follow-up questions. Referrer survey fielded. Second site day if the theatre list warrants it
Week of 28 September	Analysis. Case economics built. Funnel quantified. Findings tested back against you on anything that looks wrong
Week of 5 October	Report delivered, then a working session to walk through it and agree what happens next

If a theatre list is running on a day other than Thursday, tell us and we will come back for it, because watching one list is worth a week of description.

What we need from you

- **One point of contact** with the authority to chase people, ideally whoever runs the clinic day to day
- **The nine priority items** in the information request, before Thursday
- **A note to your team** ahead of the visit saying we are coming, why, and that this is not about individuals. It lands better from you than from us
- **Access on the day** to records, the theatre, the sterile room and the implant store, and thirty minutes each with the people listed above



- **Your referrer list**, so the survey goes to real names rather than a guess
 - **Permission to look at real patient records** on site, under confidentiality, for the ten-case trace.
- Nothing identifiable leaves the building

What you get

1. **The Osteon Organisational Audit Report.** What we found across every area above, evidenced, with what each finding costs you and what it would cost to fix, ordered by what to do first.
2. **The case economics model.** Contribution per case by procedure type, and the sensitivity of each to implant cost, theatre utilisation and price. This is the tool you keep and use for every pricing decision afterwards.
3. **The funnel and the referral picture.** Where patients are lost, what it is worth, and what the referring doctors said.
4. **A readiness verdict on publishing your outcomes.** Whether the record supports it today, what it would take, and what the numbers would say.
5. **A working session** to go through all of it and decide what we build next.

What this is not

It is not a regulatory inspection, and we have no power to report you to anyone. It is not an audit of your surgical judgement; where clinical practice is discussed it is about systems around the care, not the care itself. It is not a valuation, although the case economics will tell you a great deal about what the business is worth. And it is not a document written for a file. It is written for you, and for whoever you decide to show it to.

Consult for Africa / hello@consultforafrica.com / +234 913 813 8553 / consultforafrica.com